DR Care Solutions

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Life Plan™

This Life Plan™ is completed for all care needs assessments undertaken by a consultant from DR Care Solutions. It's used as a guideline as to the care options available for a specific client(s). It is a central place to capture your preferences around future ageing related care, types of support and living arrangements.

CLIENTS NAME: **PAMELA GARLING**
DOB: **16.08.1957 (67 years of age)**
CLIENT ID: **DRCS 0539**

Issues covered:	**Background and relevant history;** Christian Garling (Pam's stepson) reached out to Danielle from DR Care Solutions on Friday 24th January 2025 to discuss Pam's ageing care and living journey as there has been a noticeable decline in her cognitive functioning in recent months. Christian's father, Kim Garling is Pam's primary carer and has developed a degree of carer stress providing care to her 'at need' 24 hours a day, 7 days a week. Christian and Kim requested a Teams call with Danielle on Thursday 6th February 2025 to understand how DR Care Solutions could assist with planning ahead and options available. Kim arranged for Danielle to meet with him and Pam on Wednesday 26th February 2025 at their apartment in Killara. Kim welcomed Danielle into the apartment. Pam was very friendly and thought she had met Danielle before, however she hasn't. Kim has been known to Danielle and her family for more than three decades. Danielle explained to Pam that the reason for the assessment being undertaken is to develop a life plan™ should something happen to Kim, or if he is unable or unavailable to continue caring for her. Kim and Pam recently (October 2024) relocated apartments in the same block of units from upstairs to downstairs. The layout is the same. **Current health, cognition and physical (mobility) condition;** Pam is physically healthy. In 2019, she underwent a right hip replacement due to significant pain in her hip where she had 'bone on bone'. She recovered well from that surgery however her cognition was already deteriorating at that time. Kim advised he has noticed more of a decline since the general anaesthetic and that surgery.
• Physical health including mobility, continence, • Cognitive health • Sensory (sight, hearing, oral, feet) • Activities of daily living • Emotional & mental health • Risk assessment of the home and modification recommendations • Medications, allergies & vaccinations • External assistance - including Allied Health • Social background, networks and Relationships	

• Likes & dislikes • Cultural linguistic and Spiritual needs • Estate Plan/ Advice and Life Ready Register • Financial Plan/ Advice • My Aged Care • Care and support considerations	Pam has dry skin on her face however it's not eczema. She has always had some digestive issues. She recently had a colonoscopy which has come back clear. Pam has been seen by a Psychiatrist, Dr Joanne Wright who specialises in Aged Psychiatry. She diagnosed Pam a few years ago with early on-set dementia stating she's at level 3 of the highest level 4 for dementia. She has also been seen by Associate Professor Jean-Pierre Halpern who is a Clinical Neurologist. Pam failed her basic cognitive testing with him. Pam has no insight into her current situation and is aware that something 'isn't right'. She appears to confabulate due to her poor memory and has word finding difficulties (dysarthria). She shows frustration when attempting to articulate what she wants to say and can't get it out. She doesn't believe she needs support however when Kim explained he does 'everything for her', she agreed. She is quite worried about Kim not having the ability to care for her long term. Pam has become very childlike in her approach and responses. Kim doesn't leave Pam except for the shortest periods of time and if he does, he double locks all the doors to ensure she remains safe. We strongly suggest this doesn't occur as it's regarded as a restraint. Instead, we recommend that Kim (or a carer) takes Pam with them or remain with her if Kim is out. Pam hasn't been driving for three years however she maintains a driver's license and uses it as identification only. **Clinical needs:** Pam's skin is intact. She hasn't any bruising, yet it appears dry. She has no open wounds. Pam remains fully continent of bowel and bladder. Kim mentioned Pam rarely wears underpants. It's easier for her to manage toileting at home without them on. Currently, Pam has no other clinical needs. She's not diabetic nor incontinent. This may alter in the future. **Sensory needs:** Pam wears glasses for reading however her visual field has altered because of the cognitive decline. Kim explained that Pam will query 'where the orange juice is' when it's right in front of her. She reads her kindle over and over (on the same story for several hours a day) and becomes frustrated if it runs out of power. Pam has all her own teeth and visits a dentist on a regular basis. She had fillings recently. Oral hygiene is important for those living with dementia. She cleans her teeth without prompting. Pam's hearing should be tested as several times she missed the question. There was no noise in the room. We suggest a hearing test is undertaken with possible fitting of hearing aids to keep Pam socially connected when out in the community.

	Pam used to see a professional podiatrist however hasn't for a while. She has manicures and pedicures locally. We suggest a professional podiatrist be visited every few months for nail and foot health as well as assessment of Pam's balance. It's wise to check with Pam's GP to see if she can obtain five free visits through her GP Health Plan. **Activities of daily living;** Kim has been more involved in supervising and supporting Pam's activities of daily living including providing standby supervision while she's showering (in the afternoon or early evening) and drying. The shower floor never becomes slippery (from shampoo and conditioner) as Pam doesn't wash her hair however, she wets it. Every four to five weeks she visits the same hairdresser she's seen for years. Kim supports her with dressing. He mentioned that she has the same five dresses which she wears although she has wardrobes full of clothing and chooses not to wear anything apart from the 'dress'. Pam can no longer prepare meals and needs to be encouraged to eat appropriate food. Her diet has altered significantly with her putting on weight. She is requesting a diet of high salt, high sugar and processed foods. Currently, the diet and meals are Kim's main frustration with her. She has put on weight and will continue to do so if something doesn't alter. They visit local cafés for brunch or lunch most days. Pam doesn't drink alcohol. All Pam wants to eat calamari and chips, fish and chips, ice-cream, chocolates, orange juice and lemonade. The healthy foods she is prepared to eat is toast with tomato and avocado or a sandwich with cheese, tomato, avocado and beetroot (for dinner nearly every night over the last four weeks). She's refusing to eat chicken, eggs, spaghetti, pastas and no spices or curries. These are foods she previously ate. She advised she gets indigestion and requires 'Quickeze' tablets if she eats spicy foods. Pam's unable to entertain herself, take any medication, prepare meals or answer the phone. Pam wakes around 6.30am and sleeps well at night. She spends most of the day reading her Kindle (reading the same book over and over: 'The Good Father'). When we asked her to explain what her book was about, she had difficulty articulating this, even with Kim's prompts. She watches television from 3pm to 9pm without a gap. She watches game shows and continues to ask Kim when 'wheel of fortune', 'The Chase' or 'Seinfeld' is on. **Emotional and mental health;** Pam is extremely attached to Kim. She is overly anxious and emotionally labile. At times she becomes cranky due to frustration. Often what she says, she believes is correct. It's best not to argue in those cases to avoid confrontation. Pam has a lot of anxiety however she hasn't been diagnosed with depression. Pam suffers from serious 'separation anxiety' if Kim is not in sight.

	Kim appears tired and at times becomes frustrated too. He needs to have time away from Pam to enable him to rejuvenate to continue to care and support her. Several occasions Pam articulated how wonderful Kim is to her and 'he's the best man in the world'. She was teary throughout the assessment. **Risk assessment of the home and modification recommendations:** As Kim and Pam are renting the apartment, they haven't put up images or paintings on the walls. The selected artworks are 'leaning' against the walls currently. We suggest relocating them for the moment as they may become a trip hazard for Pam. The hallway areas are restricted for ambulating. The bathroom is accessible however a shower chair and interim rails ie: Shower Anti-Slip Grab Bar,Bathroom could be considered. We also suggest using Plug In Sensor lights LED Plug-In in the hallways and bathrooms as an alternative to turning on lights at night. Most falls take place at night and in the bathrooms. All floor rugs should be rolled up and removed to prevent any trip hazards. Kim currently 'supervises' Pam in the shower. If Pam is remaining in this apartment long term, we suggest that an Occupational Therapist undertakes an assessment to provide recommendations for any modifications and equipment needed. **Medications, Allergies and vaccinations:** Pam is fully immunised including the Flu Vaccine and most up to date Covid booster. We are unsure if Pam's had a shingle vaccination. Neither of them has contracted Covid to date. Kim advised they avoided contact with people during the Pandemic. Pam has no allergies that they are aware of. Kim advised that Pam is not on any medications at all. They tried **Aricept (Donepezil)** for a three-year period to treat the symptoms of Pam's dementia. Kim advised it didn't make any significant difference, and they discontinued its use (under medical supervision). We thought Pam was on anti-anxiety medication however Kim advised she was not on any medication so this needs to be confirmed. We haven't seen a GP Health Summary so can't confirm if there are any other medications or supplements being prescribed. If any medications are prescribed in the future, we strongly suggest that any medications and supplements are placed into a Webster-Pak® arranged by the pharmacy on a fortnightly or monthly basis. This assists clients to avoid over or under medicating which often happens to Senior Australians. It also assists if a person needs to go into hospital or care, that all the medications are in the one spot and can be collected quickly.

We have a preferred partnership with Webstercare®. You can find a Pharmacy location where Webster-Paks® are supplied. Find a Webster-Pak® Pharmacy Near You | Webstercare

External assistance: There is currently no external assistance in place to support Kim to care for Pam. We would initially suggest domestic cleaning, meal preparation, shopping, laundry, bedlinen changes, companionship and supervision to commence immediately 'fee for service' and privately funded while waiting for any Government Subsidies to come through.

Social background, networks and relationships; Pam was able to recall some of her long-term memories however Kim was there to prompt her. She was born in West Street, Blakehurst. She has three siblings. Her elder sister, Elizabeth who is 3 or 4 years older than Pam also has early onset dementia. She is married and lives near Campbelltown. She has 24-hour care in place and her husband is providing the care. They have an older son who is wheelchair bound after a workplace injury and a second son who doesn't live too far away. He shares the care of Elizabeth with his father however he works full time.

Pam had two adopted brothers, one older than her (Bruce) and one younger brother (Richard). Bruce was in the police force. Richard developed MS and Cancer and was living with his eldest son in Glebe. He and his son have both passed away in very sad situation.

Pam's parents were hard workers. They ran a removals business, Blakehurst Removals. Her father worked 'hands on' in the business. He passed away from stomach cancer. Her mother worked in the business undertaking all the administration of the business. Her father was a student at Sydney Grammar School and was highly intelligent.

Pam attended Blakehurst Primary and High School. She then attended Secretarial College and learnt shorthand. Danielle asked her if she could still manage it and she advised she couldn't. She worked for an accounting firm for a few years and then an Insurance Company.

When Pam was younger, she was quite creative; she made hand crafted wooden boxes, she loved to knit and was great with her hands. She no longer undertakes any creative pursuits.

Pam was married to another gent (Bill Price) for about 18 months. She had no children with him. The relationship deteriorated and she left him.

In 1983, Pam met Kim through the Law Society. Kim had set up the Young Lawyers Association and Pam was the secretary there. They developed a relationship and moved together in 1984.

	Kim had two children from a previous marriage to Christine (she currently lives in Bowral and has always been an Educator and lecturer in Education). Their two sons, Christian (in early 50s) and Jeremy (late 40s) lived with Kim and then with Pam when Christian was 7 years of age. Christian is a lawyer and has always worked in the Insurance industry. He lives in Killara not far from Kim and Pam. Jeremy went to NIDA and runs a very successful big events business. He is rarely in Sydney and travels the world for his business. When he's in Sydney, he's based in Bondi. Pam and Kim then had two other sons, Kim and Sam who are currently both in their 30s. They were like the 'Brady Bunch' and ran a very busy life with four sons. They were living in Chatswood when the four boys were living at home. Pam and Kim's son, Kim lives in Port Moresby, New Guinea. He works for the High Commission there. He has a young daughter, Charlotte (2½ years). Sam works for Deloitte in the IT area and has a daughter, Zoe (3 years). He lives in Artarmon. Pam's face lit up when we spoke about the granddaughters as she's only ever had 'boys' in her life. Pam's social network is diminishing. Kim is doing his best to continue engaging with their friendship group with regular interactions and catch ups. Pam has a few friends (from her children's schools and her earlier life) who continue to keep in touch with her. She has one significant friend, Pam and another friend, Hayley who keep in regular contact with her. All their friends are aware of the changes and supportive to Kim and Pam. **Likes and dislikes:** Pam has become a sweet and junk food eater. Her preference is to eat highly processed foods. Ice-cream is something she has most days and requests it at every meal. Pam enjoys music however rarely listens to it these days. She used to enjoy Andre Rieu and Andrea Bottcelli. She loves watching television. Pam can walk a couple of hundred metres only. Her fitness has deteriorated. Kim is encouraging her to walk daily. This is something a carer could spend time doing with Pam after driving to an area which is flat and open (balmoral beach promenade, garden centres and the like). Pam used to enjoy watering and maintaining the plants on their balcony however she's lost interest in that now and Kim maintains them. She enjoys going to Garden Centres (Eden Gardens or Flower Power) as an outing.

	Pam likes animals. They don't have any however Christian and Sam have dogs. Pam advised that Kim, her children and grandchildren bring her joy as well as her regular television programs and icecream! **Cultural, linguistic and spiritual needs:** Pam is Catholic, and they used to regularly go to mass. Kim was on the parish council and used to read at Mass. Pam speaks English and no other languages. She has no other cultural or spiritual requirements. **Personal alarms and monitoring;** Given the fact that Pam is never left alone, she won't require a personal alarm or pendant. We suggested to Kim that **he investigates** an Apple Watch (series 7 and above) or something similar **for himself**. If something happens to Kim unexpectedly, Pam wouldn't have the insight to know what to do in an emergency. The watch would need to have **GPS tracking and falls detection** with five emergency contacts listed (including the emergency services). A locked box with a spare set of keys should be located somewhere in the common areas should entry be required. The watch should be worn in the shower and while sleeping and charged first thing in the morning when Kim is fresh and feeling well. **Estate planning;** Kim is a solicitor. He has Powers of Attorney and Enduring Guardianship in place. We have received e-copies of these. Only if Pam needs to relocate into assisted living / residential aged care, will we need to send them out. We suggest that Kim reviews his estate planning documents on a regular basis, particularly if Pam is listed as his attorney or guardian. The four main documents we recommend are a will; Powers of Attorney (for financial decisions should a person lose capacity), Enduring Guardianship (for any health, care or accommodation decisions should a person lose capacity) and an Advanced Care Directive. For more information on Estate Planning for families, check the Estate Planning piece of the puzzle here: Aged Care Puzzle | DR Care Solutions | **Advanced Care Directive:** Below we include a special DR Care Solutions link to Touchstone Life Care for building an Advanced Care Directive. You are welcome to use this or ask your GP for a paper-based document if it's preferred. This is usually stored in the client's GP Health Summary and hospital records should something occur, the medical profession adheres to the requests if the person is unable to communicate their wants, needs and wishes. Touchstone Life Care - A DR Care Solutions Partner

	Life Ready Register: For DR Care Solutions clients, we provide a copy of the Life Ready Register, a depository to keep all your affairs in one place. We suggest that you download this and start completing it. No matter what age, everyone can commence building this out and sharing it with their trusted network, their Powers of Attorney and Enduring Guardians as well as their Executor of their Will. We often find the adult children of our clients using this document. Life Ready Register ™ - Free Aged & Disability Care Industry Resource **Funeral arrangements:** We suggest considering organising Funerals well before it's required. We work very closely with Funeral Directors (from the Family Business Sector) and would be more than happy to introduce you to businesses that cover your area if requested. Be mindful of payment plans when it comes to funerals, as often they cost more than a standard funeral. Preplanning eases your family's burden during an emotionally difficult time. If you have specific questions, feel free to ask. **Financial planning;** Kim and Pam are self-funded retirees. This doesn't mean that they can't access Government Funded Services. Some services are not means tested (Commonwealth Home Support Program which is entry level access to services within My Aged Care) and others are means tested (Home Care Packages, Residential respite care and permanent residential care). For more information on how the Government Support works, please review this section on our website. Government Support | DR Care Solutions. **My Aged Care;** We understand that Kim arranged an over the phone assessment with My Aged Care in November 2024. He is still waiting for the codes for residential respite care and residential permanent care. We suggest phoning My Aged Care and chasing these referral codes up as we can't consider residential care with out them. We also suggest that a **Home Care Package** is investigated and considered. Based on our assessment, Pam would likely be assessed at a level 3 or 4 Home Care Package given her current care needs. Please note that a Home Care Package is means tested. We believe that Kim has been provided with codes for entry level Commonwealth Home Support Program for Cottage Respite (for a few days) and flexible respite (in the home) which will be minimal assistance for Kim to have a decent break. It might also be very disruptive for Pam to relocate into and out of care and back home again (ie: to Cottage respite and / or residential respite). We only suggest one move for those living with dementia and cognitive decline. Kim has not investigated providers yet. He is unsure if this is the right support for them currently.

	Commonwealth Home Support Program (CHSP) services are not means tested however the client must contribute towards their care and services. Different providers provide different services through the CHSP. **Care and Support considerations**: We have spoken with Kim and Christian to determine whether they think that Pam would be accepting of support from a carer for (initially) at least a couple of days a week to take her out into the community or undertake activities of interest in her home to allow Kim to have a break either outside of the home or at home (if Pam is taken out). We are waiting to hear back from Kim as to whether this should be trialled. If Government support **isn't** forthcoming, then privately funded fee for service may need to be set up in the meantime. From our 38 years' experience in the Aged Care industry, we believe there are two viable options currently available for Pam: **Option A** – Introduce and accept care and support at home for between **2 and 5 days a week for up to 8 hours** to allow Kim some decent respite. We envisage that this be trialled for 3 to 6 months to see how Pam adjusts to having the support, in addition to the care and support provided by Kim. If Pam refuses to have the support or it doesn't work out, then Option B needs to be seriously considered before Kim becomes exhausted and has it affecting his own health and wellbeing. **Option B** – Relocating Pam into care for respite in residential aged care setting or a group home with a view to permanent care. Finding the right home for Pam will be imperative (and it will give Kim peace of mind knowing she is being well cared for and safe). It isn't just finding her a room, it's finding her next home. The selected home will need to have top quality and skilled care staff for those living with dementia. We would request their experience working with those clients living with **'early onset dementia'.** We envisage there would need to be a younger cohort living there with similar interests to Pam. This will be essential for an excellent outcome.
Goals (what does the client wish to achieve): ● Options	Kim expressed the wish for Pam to **remain at home** with quality care and support in place (at this time). We explained that getting a quality home care provider (with a Case Manager overseeing her care) having a supportive team around her to ramp up hours and days, 'as needed' is what needs to occur.

● Preferences ● Ideal Outcome ● Carer Profile	If Pam has the capital resources to remain at home with support, she should be given that choice, although it's very expensive to have 24-hour care in the home. We have several clients with 24+-hour care per day. Care and support can be provided from 3 hours per visit to 24+ hours a day. The client needs to be assessed as to where they are in their ageing journey, whether this is realistic, financially viable and the best solution for the client and their family. As outlined above, as an **alternate option**, Pam could move into residential aged care or a group home for a period of respite (usually a minimum of three weeks) with a view to permanent care. **Carer Profile:** We envisage strong, caring, energetic, empathetic and highly skilled **female carers** be employed to support Pam to remain at home for an interim period. Intelligent, well-travelled and educated carers, who can converse with her would be ideal. Someone with a creative background and good initiative to engage and divert her attention will likely enhance her quality of life.A care team could be gradually built up over time. A team of two or more carers would be a great support to Pam 'as needed' physically, emotionally, and intellectually particularly as her condition progresses. Carers from the local area would be preferred and with a clean driver's license and well-maintained vehicle. Homemaking skills would be advantageous to maintain the home including changing the bedlinen on the bed, undertaking the washing and laundry, cleaning and drying the bathroom after showering yet have the necessary 'carer' skills to help Pam in the shower, dressing, undressing, toilet, bed, transfers 'as needed'. Assistance with shopping and carrying heavy groceries, meal preparation and cooking too. Carers who have clinical knowledge of someone living with early onset dementia and cognitive decline who can identify any changes in Pam's condition would be helpful to relay back to their Case Manager (Generally a Registered Nurse). We expect that the carer would outline a daily schedule of the visit and activities they could attend to, ***in advance*** to allow for Kim to arrange his own hobbies, appointments and social engagements around them.
Actions required: (for the client to achieve these goals)	1) **Consider arranging immediate in-home care and support**. We can obtain quotes from quality providers. We will need estimated days and times that Kim is considering for Pam initially.

	2) Request a **new ACAT assessment** (from My Aged Care) as Regional Assess Services (RAS) was conducted over the phone and is inadequate for Pam. As part of ACAT re-assessments, please obtain additional codes for **a Home Care Package as well as Residential Care (respite and permanent)**. These are the codes that unlock the funding to use Government Funded services as well as the ability to relocate into care. 3) Review existing **Estate Planning Documents** including wills. Establish back up or alternate Powers of Attorneys, Enduring Guardians and Executors and a copy of this life plan™ should be provided to Pam's Attorney(s) and Guardian(s). 4) Development of an **Advanced Care Directive with** specific emphasis on healthcare and accommodation decisions. 5) Complete the DR Care Solutions, **Life Ready Register** and provide the completed document to their Next of Kin and Executors. Information provided in this life plan™. 6) Consider setting up a **prepaid funeral** in event of one's passing. Please advise Danielle for an introduction to one of our Strategic Partners (Family Business) if interested. 7) The use of **personal alarms or monitoring** devices for emergencies such as falls. Information has been provided in this life plan. This is for Kim rather than for Pam, to give Kim peace of mind should something happen to him, someone else can be alerted. Pam hasn't the insight to know what to do. 8) Pam to have a **hearing test** and obtain hearing aids if deemed necessary. 9) Pam to have her **eyes tested** regularly and obtain new glasses if deemed necessary. 10) Pam to visit the **GP, Geriatrician and Neurologist** and any other medical specialists 'as required' to manage the progression of her dementia and other health issues. 11) Pam to visit a **professional Podiatrist** to maintain foot health, nail health and aid balance (Check with GP as may be able to obtain five free visits per annum via the GP Health Plan).
Planning ahead and Implementation:	**Planning ahead:** Should something happen to **Kim,** the family would need to arrange immediate additional domestic support (including cleaning, shopping, cooking), companionship and care for **Pam**. Financially, if Pam could sustain 24 /7 care at home in the long term then this is a good option.

	This needs to be confirmed by Pam's Financial Advice team. We would work with the family to determine if this is the right option for Pam at that time. Alternatively, we (at DR Care Solutions) could look to relocate Pam quickly and professionally into an assisted / supported living environment (Residential Aged Care or a Group Home) to ensure she is safe and well cared for and has social connection, particularly if Pam's cognitive and physical condition deteriorates significantly (with double incontinence developing, behavioural issues or continual falls). We recommend that anyone with dementia only has ONE move from their own home as it's stressful at the best of times, let alone being an older Australian. It can dramatically increase the progression of any disease if they need to relocate more than once (ie: from home, into a retirement community into residential aged care). **Implementation:** When the decision is made to implement this plan, we suggest that DR Care Solutions is engaged to select and introduce a suitable approved, pre-vetted Home Care Provider to support Pam remain at home. We will introduce up to three pre-vetted quality home care providers (approved by the Government). One provider may have the perfect carers and support for them and therefore only one provider will be introduced. We need to ensure her care needs are being met including occupation, physical activity, community access and socialisation. Consistency of carers is key to ensure both feel safe and comfortable. A team of at least **two skilled carers** should be able to cover the initial days needed. We always like to have at least two carers in a roster to ensure there is always someone who knows the client's routine and where things are kept, particularly if one carer is unwell or unavailable. A Case Manager is generally allocated to the family from the Home Care Provider, and they will visit Pam regularly to ensure there is complete oversight with the team of carers and ensure that Pam has the appropriate support at that time. A Case Manager is generally a Registered Nurse who can clinically quickly pick up any issues in relation to their condition, behaviour or clinical care needs. The rates being charged by providers 'fee for service' **range from $76 to $88 per hour** Monday to Friday for fully insured and employed carers (Minimum charge is generally 3 hours per visit). Weekends and public holidays incur surcharges.

	Review of the hours and her situation should be undertaken monthly by the Case Manager and relayed back to the family to ensure the right amount of care and support services are taking place for her at any given time. Government funding can be added to the mix of funding once it comes through. A reminder that Home Care packages through the Government are means tested. Some providers offer a LIVE-IN service (24 hours a day) with one carer per day. The client needs to be assessed (by the provider) as to whether they need support overnight or just need someone on site in case of emergency. We can discuss this option, if or when it is required. Other providers only offer coverage of the 24 hours in 8 hour or 12 hour rotating shifts. We will guide and support you through this process to get the right outcome for Pam at any given time. As outlined above, should Pam's condition deteriorate significantly to a point where she can't be cared for at home safely, we can be **re-engaged** to find the right Aged Care Home (or Group Home) for respite with a view to permanent care. We run a comparative platform report which brings up the top 10 Aged Care homes in a certain area. We work closely with the family to find the right home for the client and tour up to four or five in a particular location. It's not just locating a bed for a client, it's their next home. We personally ensure that the selected home is appropriate for the client's needs, wants and wishes within their financial capacity to afford the home and that they are clinically accepted by the home. It's not just the family determining the home We undertake all the groundwork, pre-vetting and due diligence of the aged care homes and remove the pressure and stress out of finding the right home at an already emotionally charged time. We walk alongside you for as long as the client needs care or is in care.
Resources:	Explore Aged Care Puzzle, a resource designed to guide you through every step of the aged care journey. With our step-by-step puzzle solution, you can confidently make informed choices at your own pace. Just pick your first Aged Care Puzzle piece, and you're on your way! Aged Care Puzzle | DR Care Solutions | DR Care Solutions

Who will be involved: (Guardian, solicitor, financial planner/advisers, family members)	• Kim Garling, Christian Garling and other family members • Danielle Robertson from DR Care Solutions • Financial adviser / Accountant • Estate Lawyer (review of all estate planning documents) • GP, Geriatrician, Neurologist and any other medical specialists
Review of Life Plan and the date the Family need to determine next steps:	Wednesday 12th March 2025

Signature (Consultant):  Date: Wednesday 5th March 2025

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